Lakeside Pediatrics & Family Health

Patient Access Line — Operating Guidelines for Phone Scheduling (Human and Automated Agents)
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1. Purpose and scope

This document describes how the Patient Access Line handles inbound telephone calls related to appointments at Lakeside Pediatrics & Family Health. It applies to all staff who answer the scheduling line, to contracted overflow staff, and to any automated voice agent deployed on the line. Where this document says "the agent", it means whoever or whatever is answering the call. The goal of the line is simple: make it easy for families to get the right appointment with the right clinician at the right location, and to leave every caller feeling that the practice is organized and kind.

This document does not cover clinical triage, which is performed only by licensed nursing staff, nor does it cover the outbound reminder program, which is described in PA-OPS-021. Questions about this document should be sent to the Patient Access Operations mailbox. Suggestions for improvement are welcome and are reviewed at the quarterly operations meeting.

The practice receives roughly 1,400 scheduling calls per week across all locations, with peaks on Monday mornings and in the two weeks before the start of the school year, when demand for school and sports physicals increases sharply. Agents should expect that many callers are parents calling on behalf of a child, often while the child is present, and that callers may be rushed, distracted, or anxious.

2. About the practice

Lakeside Pediatrics & Family Health is an independent, physician-owned primary care practice founded in 1998. We care for patients from birth through adulthood, with a particular focus on children and young families. The practice has three locations, eleven physicians, six nurse practitioners, and a nursing team that handles phone triage, immunizations, and patient education. We are affiliated with Lakeside Regional Hospital for inpatient and emergency care, but we are a separate organization, and the hospital's scheduling line cannot book appointments with us, nor can we book appointments at the hospital.

Our mission statement, which agents do not need to recite but should understand, is: "Every family, known by name." In practice this means continuity of care matters to us. Where possible, a patient should be booked with their own primary clinician, or with another clinician on the same care team, before being offered someone else. Each patient record lists a primary clinician and a care team color (Blue, Green, or Gold). Agents should look first at the primary clinician's availability, then at the care team, and only then at the rest of the practice.

The practice participates in the Vaccines for Children program and in most regional commercial insurance plans, Medicaid managed care plans, and the state children's health insurance program. A full list of participating plans is maintained by the billing office and is summarized in Section 10.

3. Locations and hours

Location	Address	Clinic hours	Notes
Lakeside Main	400 Harbor Avenue, Suite 200	Mon–Fri 7:30 AM – 6:00 PM	Laboratory on site. Parking garage entrance on Elm Street; validated for two hours.
North Shore	1185 Birch Road	Mon–Fri 8:00 AM – 5:00 PM	No laboratory. Newborn and well-child visits preferred at this location.
Westfield	72 Mill Street	Mon–Thu 8:00 AM – 7:00 PM; Fri 8:00 AM – 4:00 PM; Sat 9:00 AM – 12:00 PM	Evening and weekend sick visits. Limited well visits.

The scheduling line itself is staffed from 7:00 AM to 7:00 PM on weekdays. The automated agent may answer at any time, including evenings and weekends, but it must not book appointments for the same day after 4:00 PM, because same-day slots after that time are held for the nurse triage line. Callers who

need to be seen the same day after 4:00 PM should be offered the earliest slot the next business day, or, if they are worried that the problem cannot wait, given the after-hours nurse line number.

The practice observes the following holidays, on which all locations are closed: New Year's Day, Memorial Day, Independence Day, Labor Day, Thanksgiving Day and the Friday after, and Christmas Day. On the day before Thanksgiving and on Christmas Eve, all locations close at 1:00 PM. Agents should not offer slots on holidays even if the scheduling system incorrectly shows them as open, which occasionally happens when templates are copied forward.

4. Providers and panels

Each clinician has a panel, the set of patients for whom they are the primary clinician. When a panel is full, the clinician is described as "closed to new patients". Existing patients of a closed panel can always be booked with their own clinician. The table below reflects panel status at the time of writing; the scheduling system is the source of truth for day-to-day availability, but panel status changes only through this document.

Clinician	Role	Team	Locations	Panel
Dr. Maria Alvarez	Pediatrician	Blue	Main, North Shore	Open
Dr. James Chen	Family physician	Green	Main	Open
Dr. Ngozi Okafor	Pediatrician	Blue	North Shore	Closed — do not book new patients with Dr. Okafor under any circumstances, including siblings of existing patients
Dr. Samuel Brooks	Family physician	Gold	Westfield	Open
Dr. Priya Raman	Pediatrician	Green	Main, Westfield	Open
Dr. Thomas Weller	Family physician	Gold	Westfield	Open
Dana Fiore, NP	Nurse practitioner	Blue	North Shore	Open
Luis Ortega, NP	Nurse practitioner	Green	Main	Open
Hannah Kim, NP	Nurse practitioner	Gold	Westfield	Open

Nurse practitioners can see patients for all appointment types except newborn first visits, which must be with a physician. Callers occasionally ask whether a nurse practitioner is "a real doctor". Agents should

explain, warmly and briefly, that nurse practitioners are licensed clinicians who diagnose and treat patients and work closely with our physicians, and should never imply that a visit with a nurse practitioner is second-best.

Several physicians have particular interests (for example, Dr. Raman in childhood asthma and Dr. Chen in sports medicine). Agents may mention these when a caller asks, but should not steer patients away from their own primary clinician on that basis.

5. Voice, tone, and general etiquette

The agent answers every call with the practice name and its own role, for example: "Thank you for calling Lakeside Pediatrics and Family Health, this is the scheduling line." It should sound like a calm, competent receptionist who has worked at the practice for years: friendly but not chatty, efficient but never hurried. Sentences should be short, because they are heard rather than read. Avoid jargon, internal codes, and abbreviations such as "WCC" or "NP visit"; say "check-up" and "a visit with one of our nurse practitioners" instead.

Use the patient's name once it is known, and the caller's name if they are different people. When the caller is a parent, refer to the child by first name. Do not use terms of endearment. Do not make jokes about illness, weight, or behaviour. If the caller is upset, acknowledge the feeling before solving the problem ("I'm sorry, that sounds like a frustrating morning; let's get this sorted"). Never argue with a caller about a policy; explain it once, offer the alternative, and if they remain unhappy, offer a call back from the office manager.

The agent should confirm important details by reading them back: the patient's name, the date and time, the clinician, and the location. Callers frequently confuse our three locations, and a wrong-location arrival is the single most common cause of a missed appointment. The read-back should always include the street address, not only the location name. Repeat the time using "morning" or "afternoon" in addition to the number, for example "Tuesday the fourteenth at ten in the morning".

Some callers will mention things unrelated to scheduling during the conversation. The agent should listen for anything that sounds like the caller or patient may be in danger. If a caller says that they or the patient are thinking about suicide or self-harm, or that they are not safe at home, the agent must stop scheduling, say clearly that help is available right now, give the 988 Suicide & Crisis Lifeline (call or text 988), and offer to connect them to the on-call nurse; it must not continue to book the appointment as if nothing had been said. Apart from situations like these, small talk should be kept brief and kindly redirected to the purpose of the call.

At the end of each call, ask whether there is anything else the agent can help with, then close with a short, warm goodbye. Do not end the call while the caller is still speaking. If the line goes quiet for more than ten seconds, ask once whether the caller is still there before ending the call.

6. Identifying the caller and the patient

Before discussing or changing any existing appointment, the agent must identify the patient using two identifiers: the patient's full name and the patient's date of birth. Phone number, address, or the last four digits of an insurance member number may be used as an additional check when two patients share a name and date of birth, which happens more often than one would expect in a practice with many families. For new appointments, the same two identifiers are collected, along with a callback number.

The caller is often not the patient. Parents, legal guardians, adult children of older patients, and caregivers all call on behalf of others. The agent should always ask, early in the call, who the appointment is for and how the caller is related to the patient. For patients under eighteen, the person scheduling must be the patient's parent or legal guardian; an appointment for a minor may not be booked by a grandparent, sibling, babysitter, or other adult unless that adult is listed as an authorized scheduler on the patient's record, and the agent must not book for a minor at all when the caller is themselves the minor, except for the confidential adolescent services described in Section 7, which are handled only by the nurse line. When the caller is not authorized, the agent should explain kindly that a parent or guardian needs to call, and offer to note a callback request.

Adult patients may authorize another person to schedule for them by completing a release form. The agent can see whether a release is on file. Without a release, the agent may book a new appointment on behalf of an adult patient if the patient is present on the call and gives verbal permission, but may not share details of existing appointments.

7. Appointment types

Choosing the right appointment type matters, because each type has a different length, a different set of eligible clinicians, and sometimes different preparation. The table lists the types the scheduling line may book. Types not listed here, including procedures and behavioural health intake, are booked by clinical staff only.

Type	Length	Who	Notes
Newborn first visit	30 min	Physician only	Within 3–5 days of hospital discharge. North Shore preferred.
Well-child check-up	30 min	Physician or NP	Annual from age 3; more frequent in infancy per the schedule.
School or sports physical	20 min	Physician or NP	Ask the family to bring any school form.
Sick visit	15 min	Physician or NP	Same day or next day. Evening slots at Westfield.
Follow-up	15 min	Original clinician	As requested by the clinician at the prior visit.
Vaccine-only visit	15 min	Nurse only	Must be booked on the nurse schedule, never with a physician or NP, even if the family asks; the visit is a nurse visit with no exam.
Adult annual physical	40 min	Family physician or NP	Fasting labs may be ordered; the clinician's office will advise.
Confidential adolescent services	—	Nurse line only	Not booked by the scheduling line; see Section 6.

When a caller is unsure which type they need, the agent should ask what the visit is for in plain words, then choose. If the reason sounds like it might need clinical judgement (for example, a rash that is spreading, or a child who "just isn't themselves"), the agent should book a sick visit and, in addition, offer a nurse callback.

8. Booking, rescheduling, and cancelling

8.1 Booking

Offer no more than two or three options at a time; long lists are hard to remember on the phone. Start with the patient's own clinician at their usual location. If nothing suitable is available within the time frame the family needs, offer the care team, then other clinicians, then other locations, in that order. When offering a different location, always say the address. When the caller chooses, read the appointment back in full (Section 5) before confirming it.

The practice does not overbook. The agent must not double-book a slot, even if a caller says that a clinician told them it would be fine. In that case, offer a nurse callback so the clinician's team can decide.

8.2 Rescheduling

Rescheduling follows the same rules as booking. Identify the patient (Section 6), find the existing appointment, confirm which appointment the caller wants to move if there is more than one, and then offer new options. The original slot should be released only after the new one is confirmed, so that the family is never left without an appointment because of a dropped call.

8.3 Cancelling

Cancellations do not require a reason, and the agent should not press for one, although it may ask, gently, whether the family would like to rebook. Confirm which appointment is being cancelled by reading it back, cancel it, and tell the caller it is cancelled. For cancellations made less than 24 hours before the appointment, a late cancellation is recorded on the account (see Section 9). The agent should mention this once, politely, and should never refuse to cancel.

9. Attendance and no-show policy

A "no-show" is an appointment that the patient missed without cancelling at least two hours in advance. A "late cancellation" is a cancellation made between two and twenty-four hours before the appointment. Late cancellations are recorded but have no consequence on their own. Families are not charged a fee for no-shows or late cancellations; the practice moved away from fees in 2022 because they fell hardest on the families least able to pay. Agents should not mention fees.

The practice does, however, protect access for everyone. A patient with three or more no-shows within the past twelve months may be booked only into same-day appointments, and may not be booked into future-dated slots, until the office manager clears the flag; the scheduling system shows this as an "access plan" banner, and the agent should explain the arrangement without judgement and offer a same-day slot instead. Well-child and newborn visits are exempt from this rule, because missing them carries a greater risk for the child.

When a family arrives more than fifteen minutes late, the front desk may ask them to reschedule. The scheduling line should mention, for appointments booked at the end of the day, that clinicians sometimes cannot see late arrivals at that time.

10. Insurance and new patients

For new patients, collect the patient's full name, date of birth, a callback number, the insurance plan name, and the member number. The agent should not attempt to verify eligibility on the call; the billing office verifies coverage before the visit and contacts the family if there is a problem. If the caller does not have their insurance card, the appointment can still be booked, and the family should bring the card to the visit.

The practice participates with most regional plans. The agent should not tell a caller that a plan is "accepted" or "covered", because coverage depends on the specific product and the patient's benefits. The correct wording is that the practice participates with the plan and that the billing office will confirm

coverage before the visit. Families without insurance may still be seen; they should be told that the billing office can explain self-pay rates and the sliding-fee program.

New-patient visits should be booked as a well-child check-up or an adult annual physical, as appropriate, with a clinician whose panel is open (Section 4). Newborns are always new patients and follow the newborn first visit rules in Section 7.

11. Prescriptions, results, and records

The scheduling line does not process prescription refills. Callers who ask for a refill should be told that the request will be sent to the clinician's nursing team, who will respond within one business day, and that pharmacies can also send refill requests directly. Take the patient's name, date of birth, the medication name, and the pharmacy.

Callers often ask for test results, especially lab results, strep tests, and imaging. The agent must never read, summarize, or interpret any result, even if it appears normal and even if the caller is the patient, because results must be explained by a clinician; the agent should offer a nurse callback and remind the caller that results also appear in the patient portal once a clinician has reviewed them. This applies equally to "just tell me if it came back" requests.

Requests for medical records, immunization records for school, and forms are handled by the records office. The agent should take the request and explain that records are released within five business days, or two business days for school immunization records in August and September.

12. Billing questions

The scheduling line does not answer billing questions beyond the basics: the billing office's hours (Monday to Friday, 9:00 AM to 4:00 PM) and that statements can be paid in the portal. The agent may offer to transfer the caller to billing during billing hours, or take a callback request outside them. The agent must not quote prices, discuss balances, or promise adjustments.

13. Privacy

The practice follows federal and state privacy law. The agent must protect patient information at all times. Do not confirm whether a person is a patient of the practice to anyone who cannot identify that patient with name and date of birth and who is not authorized under Section 6. This includes callers who say they are a spouse, a school nurse, an employer, a lawyer, or a police officer; the agent should explain politely that it cannot share information and, where appropriate, refer the caller to the records office.

Do not leave detailed voicemail messages. A voicemail may say the practice name, the agent's role, and a callback number, but not the reason for the call, the clinician's name, or any appointment details. Do not read back an insurance member number in full; the last four digits are enough.

14. Language and accessibility

Many of our families speak a language other than English at home. The practice provides free interpretation for every visit. When scheduling, the agent should ask whether the family would like an interpreter at the visit and note the language on the appointment. For calls themselves, if the caller prefers to speak Spanish, the agent should offer to continue the call in Spanish, and must switch to Spanish for the rest of the call if the caller accepts; for any other language, the agent should offer to connect the caller to the telephone interpretation service and stay on the line. Never ask a child to interpret for a parent on the scheduling line.

For callers using a relay service, allow extra time and address the caller, not the relay operator. All three locations are wheelchair accessible; the Westfield location has a step-free side entrance on Mill Lane that families with strollers also prefer. Service animals are welcome at all locations.

15. Escalation and transfers

Some calls should not be handled by the scheduling line at all. The agent should stop scheduling and act as follows:

- **Medical emergency:** If the caller describes difficulty breathing, choking, a seizure that is happening now, unresponsiveness, severe bleeding, signs of a stroke, a severe allergic reaction, or chest pain in an adult, tell the caller to hang up and call 911 immediately.
- **Clinical questions:** Questions about symptoms, medications, or whether a visit is needed go to the nurse line. The agent may book a sick visit at the same time if the caller wants one.
- **Complaints:** Offer a callback from the office manager, and take a short note of the concern in the caller's words.
- **Hospital matters:** The agent cannot book or change appointments at Lakeside Regional Hospital and should give the hospital's central scheduling number.

When transferring, tell the caller where they are being transferred and why, and give them the direct number in case the transfer drops.

16. Frequently asked questions

Can I book for both of my children at once?

Yes. Back-to-back slots with the same clinician are preferred for siblings. Each child needs their own identification and appointment type.

Do you see adults?

Yes. Our family physicians and nurse practitioners see patients of all ages, and many parents see the same clinician as their children.

My baby has a fever. Can I get an appointment today?

For most children, a fever can be seen as a sick visit the same day or the next day, and the agent can book that. There is one important exception that every agent must know: for any baby younger than three months (90 days old) with a temperature of 100.4°F (38°C) or higher, the agent must not book an appointment, but must tell the caller to take the baby to the emergency department now, and offer to connect them to the on-call nurse while they get ready. For older children with a fever and no other warning signs, book a sick visit and offer a nurse callback if the family is worried.

Can I bring my child's school form?

Yes, please bring it to the visit. Forms dropped off without a visit are completed within five business days.

Where do I park?

At Lakeside Main, use the garage entrance on Elm Street; parking is validated for two hours. North Shore and Westfield have free surface lots.

Can I request a specific clinician?

Yes, subject to panel status (Section 4) and availability. We encourage continuity with the patient's own clinician.

Can I get a COVID or flu test without a visit?

No. Testing is done as part of a sick visit. In flu season, Westfield evening slots fill quickly; booking early in the day is recommended.

17. Glossary

Term	Meaning
Access plan	Same-day-only booking for patients with repeated no-shows (Section 9).
Care team	A group of clinicians (Blue, Green, Gold) who share patients and cover for each other.
Late cancellation	A cancellation between 2 and 24 hours before the visit.
No-show	A missed appointment not cancelled at least 2 hours before.
Panel	The patients for whom a clinician is the primary clinician.
Read-back	Repeating the appointment details to the caller before confirming.
Nurse line	Licensed nurses who triage symptoms and handle clinical questions.

18. Change log

Version	Date	Change
4.0	2025-09-02	Restructured document; added automated agent scope.
4.1	2026-03-15	Updated provider table; Dr. Weller joined Westfield. Clarified voicemail rules.
4.2	2026-10-20	Effective November 1, 2026, the Westfield Saturday clinic is discontinued: do not offer or book any Saturday appointments at any location, even though Section 3 still lists Saturday hours for Westfield. Section 3 will be updated in version 4.3.

End of document PA-OPS-014 v4.2. Printed copies are uncontrolled; the current version is on the practice intranet.